

Mental Hygiene Law Section 9.60 -- Kendra's Law

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New York State's Mental Hygiene Law was passed in 1972 and aimed to provide services to people with mental health issues, developmental disabilities, and substance use disorders. It also addressed the rights of individuals receiving services, funding for programs, and the regulation of services (The New York State Senate, 2023). Kendra's Law, otherwise known as Assisted Outpatient Treatment, is Section 9.60 of the Mental Hygiene Law. Kendra's Law was enacted in 1999 after a woman, Kendra Webdale, was pushed off a subway platform by a man with paranoid schizophrenia who failed to comply with his medication treatment. The statute created a framework for courts to mandate assisted outpatient treatment for certain individuals with mental illnesses a way to live safely in their communities by being under mandatory supervision and treatment (Diven, 2022, p. 185; New York State Office of Court Administration, n.d.; Office of Mental Health, n.d.).

Assisted Outpatient Treatment (AOT) describes different services individuals might receive, such as case management, medication treatment, drug testing, vocational training, group therapy, substance abuse treatment, and counseling (New York State Office of Court Administration, n.d.). For a court to order AOT for an individual, several conditions must be met. Individuals must be at least 18 years of age, suffering from mental illness, and clinically determined to be unlikely to live safely in the community without supervision. They must have a history of non-compliance with treatment that has either resulted in at least two hospitalizations in the last 36 months, or violence and threats of violence toward themselves or others within the last 48 months. In addition, individuals must show that they are unlikely to participate in voluntary treatment, that AOT is necessary to prevent relapse and deterioration, and will result in positive outcomes. The process is started when a petitioner files a petition with the court for the individual in need. A physician is also required to demonstrate that alternative treatments were unsuccessful and AOT is necessary. The court then will decide if the individual fulfills the eligibility criteria for AOT (Diven, 2022, p. 186; New York State Office of Court Administration, n.d.; Office of Mental Health, n.d.). AOT orders can be granted for up to one year and upon expiring, a psychiatrist will determine if the treatment will still be beneficial to the individual and a judge will decide on the renewal (New York City Health, n.d.).

Studies have proven Kendra's Law to be effective in reducing incidents of hospitalizations, homelessness, arrests, incarcerations, and violent episodes. It has also been shown to increase voluntary compliance with treatment and reduce stress on caregivers (Diven,

2022, p. 184). A 2005 study assessing the impact of Kendra's Law found that it was successful in helping individuals to survive safely in society. Clinicians assessed outcomes after six months of treatment for 2,745 patients after six months and found a 47 percent increase in participation in case management, a 22 percent increase in medication management, and a 16 percent increase in involvement in substance abuse services. In addition, there were decreases in harmful behaviors, homelessness, and arrests (Appelbaum, 2010, p. 172). These findings have been supported by other studies conducted that have also found positive outcomes of AOT, such as improved rates of psychotropic medication possession and reduced number of hospitalizations (Van Dorn, 2010, p. 986). Assisted outpatient services have allowed people a less restrictive alternative to live safely in society.

While AOT has been beneficial to many individuals, there exists a weakness that prevents the program from being accessible to a majority of people suffering from mental illness. The statute does not take into account individuals who have mental illnesses but have yet to show evidence they are a danger to others. A high level of evidence is required that demonstrates an individual is a danger to the public based on their history in order for courts to mandate treatment. Young adults, adolescents, and older adults who develop mental illnesses in the later stages of life are less likely to be offered AOT as they have not yet shown a solid history of severe mental illness and dangerousness to others. In addition, it defeats the purpose of early preventative treatment and inadvertently leads to deterioration and worsening of mental conditions that reduces the chance of long-term recovery (Diven, 2022, pp. 187-189).

Another controversial issue associated with AOT involves HIPAA. AOT is not a voluntary service and requires disclosure of patients' medical records to determine their eligibility and course of treatment. Courts can obtain patients' medical records without their consent to determine treatment and services. Therefore, patients may not know their medical records are being disclosed to other parties (Derevlany, 2010, paras. 2-4). This can be viewed as a violation of HIPAA's Privacy Rule which gives patients the right to authorize whether covered entities can disclose their protected health information (PHI) (Clark, 2013, p. 3). However, Derevlany (2010) states that the Mental Hygiene Law allows for the disclosure of protected health information for an AOT order without patient consent (para. 5). In a court case, *In the Matter of Miguel M. v. Barron*, Miguel M. tried to appeal his AOT order that was petitioned for by Dr. Barron on the basis that HIPAA's Privacy Rule was violated because no consent was given

for disclosure of Miguel M.'s clinical records. Miguel M. argued that Dr. Barron did not follow HIPAA regulations for obtaining medical records. Dr. Barron countered that the disclosure was authorized under the Mental Hygiene Law and stated that AOT is within the scope of public health. The court sided with Miguel M. and stated that AOT does not qualify as a public health activity nor considered an exception to the Privacy Rule. They ruled that the Privacy Rule was applied to this case and that medical information acquired in breach of HIPAA is not allowed to be used for AOT orders (Clark, 2013, pp. 13-19).

Kendra's Law has helped many individuals receive the help they need by offering them a less restrictive way of treatment and at the same time keeping the public safe from acts of violence. However, a majority of mentally ill individuals are left out of receiving assisted outpatient services due to not fulfilling the eligibility requirements or have not shown they are a danger to the public. In addition, patients' medical records are disclosed without their consent for AOT which can be seen as in violation of HIPAA's Privacy Standard. This issue is addressed in the court case, *In the Matter of Miguel M. v. Barron*. The court ruled that HIPAA regulations take precedence over Kendra's Law and that medical records obtained in violation of HIPAA are not admissible for AOT orders (Clarke, 2013, p. 19). The case of Miguel M. has allowed patients to exercise their right given by the Privacy Rule and control over their medical records. Despite the drawbacks of the statute, it is also important to acknowledge the effectiveness of AOT in helping people regain control of their lives and live safely in their community. Future policies and legislation can address the drawbacks to improve assisted outpatient services.

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